

Chapter 1

INTRODUCTION TO THE PSYCHOLOGY OF SELF-ESTEEM

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ABSTRACT

Self-esteem is a central construct in clinical, developmental, personality, and social psychology. Its role in psychological functioning has been studied for more than a century. Self-esteem construct has spawned a research literature of such magnitude and richness that it is impossible to summarize. Therefore, the aim of the present chapter was to highlight the main topics in this domain. That is, the self-esteem definition, dimensionality, and components; its formation and development, and assessment; the positive and negative effects of both high and low self-esteem, the association between low self-esteem and psychopathology especially depression, the dark side of high self-esteem (e.g., narcissism); and the terror-management as an important theory of self-esteem. Then, self-esteem and both culture and demographic variables associations were reviewed, and the stability of self esteem. At last, a suggested solution to remedy the problem of social desirability in responding to the self-esteem scales was proposed.

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Through the short history of psychology, few personality constructs have received greater theoretical and empirical attention than self-esteem. In 2004, Heine and Lehman stated that over 18,000 studies investigating self-esteem have been published over the past 35 years (this is a rate of more than one publication per day!). Meanwhile, psychologists interest in self-esteem has grown exponentially over the years. Research on self-esteem has had a long prolific history in psychology.

Self-esteem is a central construct in clinical, developmental, personality, and social psychology, and its role in psychological functioning has been studied for nearly a century (Greenier, Kernis and Waschull, 1995). Self-esteem significance is often exaggerated to the extent that low self-esteem is viewed as the cause of all evil and high self-esteem as the cause of all good (Manning, Bear and Minke, 2006). In a similar vein, Hewitt (2002) stated that the conventional approach to self-esteem has spawned a research literature of such magnitude and richness that it is impossible to summarize. Therefore, the aim of the present chapter was to highlight the main topics in the self-esteem huge literature.

Everyone, must love himself or herself above or at least in equal measure to any other person or thing. This would explain the golden rule found in most religions, to “love others as you love yourself”. The coincidence among different cultures and religions regarding this essential bit of wisdom, to treat and love others, as you would have them treat and love you, is nevertheless surprising (Dolan, 2007, p. 50).

DEFINITION OF SELF–ESTEEM

Rosenberg (1965), one of the pioneers in this domain, stated that self-esteem refers to an individual overall positive evaluation to the self. He added, that high self-esteem consists of an individual respecting himself and considering himself worthy. In a similar vein, Sedikides and Gress (2003) stated that self-esteem refers to individual’s perception or subjective appraisal of one’s own self-worth, one’s feelings of self-respect and self-confidence and the extent to which the individual holds positive or negative views about self.

Self-esteem is related to personal beliefs about skills, abilities, and social relationships.

Self-esteem is also defined as a global barometer of self-evaluation involving cognitive appraisals about general self-worth and affective experiences of the self that are linked to these global appraisals (Murphy, Stosny and Morrel, 2005). By the same token, Wang and Ollendick (2001) stated that self-esteem involves an evaluation of oneself followed by an emotional reaction towards oneself. The evaluative and affective elements are present in all extant definitions and theories of self-esteem.

Brown, Dutton, and Cook (2001) distinguished three ways in which the term “self-esteem” is used: (a) global or trait self-esteem to refer to the way people characteristically feel about themselves, i.e., feelings of affection for oneself; (b) self-evaluation to refer to the way people evaluate their various abilities and attributes, and (c) feelings of self-esteem to refer to momentary emotional states, e.g., a person might say her self-esteem was sky-high after getting a big promotion, or a person might say his self-esteem plummeted after a divorce.

Perhaps the simplest definition of self-esteem is found in Webster’s dictionary, which says that “self-esteem is satisfaction with oneself”. In another edition of the same dictionary, self-esteem means “one’s good opinion of one’s dignity or worth”.

Hewitt (2002) sought to transform our view of self-esteem from a universal psychological trait and motivating force to a socially constructed emotion grounded in mood. This point of view was based on Smith – Lovin’s (1995) definition of self-esteem as a reflexive emotion that has developed over time in social processes of invention, that individuals learn to experience and to talk about, that arises in predictable social circumstances, and that is subject to social control.

For the purpose of the present chapter, self-esteem could be defined as the self-evaluation and descriptive conceptualization that individuals make and maintain with regard to themselves.

DIMENSIONALITY AND THE COMPONENTS OF SELF-ESTEEM

Self-esteem can refer to the overall self or to specific aspects of the self, such as how people feel about their social standing, racial or ethnic group,

physical features, athletic skills, job or school performance. Theorists have made many distinctions concerning different types of self-esteem, e.g., contingent vs. noncontingent; explicit vs. implicit; authentic vs. false; stable vs. unstable; global vs. domain specific. Regarding the dimensionality of self-esteem, some authors conceptualized it as a unitary global trait, whereas others view it as a multidimensional trait with independent subcomponents (performance, social, and physical self-esteem) (Heatherton and Wyland, 2003).

Branden (1969) maintained that self-esteem consists of two components: (a) to consider oneself effective, to trust in one's ability to think, learn, choose and make correct decisions, and to overcome challenges and produce changes, and (b) to respect oneself, the confidence in one's right to be happy, and the confidence that people are worthy of the respect, love and self-fulfillment appearing in their lives. More recently, Reasoner (2005) viewed self-esteem as composed of two distinct dimensions: competence and worth. On the basis of these two components, he defines self-esteem as "the experience of being capable of meeting life challenges and being worthy of happiness".

In personality psychology, there is a well-known distinction between traits and states first introduced by Cattell (1966) and elaborated by Spielberger and his colleagues (1983). Based on this distinction, some authors distinguished between trait self-esteem, i.e., stable in time as it is a part of the personality and the state self-esteem, which is more labile, being affected by events, situations, and emotions (Gilovich, Keltner and Nisbett, 2006).

Deci and Ryan (1995) distinguished between contingent and true self-esteem. Contingent self-esteem refers to feelings about oneself that result from, and dependent on, matching some standards of excellence or living up to some interpersonal or intrapsychic expectations. It is a kind of aggrandizement of oneself associated with being ego-involved in some types of outcomes and dutifully achieving them. It often involves social comparison and tends to be associated with a kind of narcissism. In contrast, true self-esteem is more stable and based in a solid and secure sense of self. Their worth would be an integrated aspect of one's self and would be reflected in agency, proactivity, and vitality.

As for the assessment of self-esteem, some authors distinguished between explicit self-esteem (questionnaires) and implicit self-esteem, i.e., the introspectively unidentified (or inaccurately identified) effect of the self-attitude on evaluation of self-associated and self-dissociated object (Greenwald and Banaji, 1995).

THE FORMATION AND DEVELOPMENT OF SELF-ESTEEM

The formation of self-esteem implies a long process. It is correlated with the formation of self-image and self-conscience. Its evolution in time involves also downfall periods especially during transition periods from one stage to another, from one status to another, e.g., in adolescence (due to the psychosomatic changes), or grand age, as a consequence of the change in status, retirement and the change in tasks and responsibilities (Orth, Trzesniewski and Robins, 2010). While self-esteem appears to decline during adolescence, it increases during young adulthood (Tsai, Ying and Lee, 2001).

The affective model of self-esteem development assumes that: (a) self-esteem forms early in life in response to relational and temperamental factors; and (b) once formed, endows high self-esteem people with the ability to promote, protect and restore feelings of self-worth (Brown et al., 2001).

Many studies have underlined the essential role of the family environment in the formation of personality especially in the early childhood (Talib, Mohamad and Mamat, 2011). Early studies by Rosenberg (1965) and Coopersmith (1967) showed that parental involvement and willingness to give adolescents autonomy and freedom are positively correlated to high self-esteem in adolescents.

The period of adolescence is important for the process of self-esteem formation. The formation of self-esteem can be stimulated, encouraged both by parents and teachers. The level of self-esteem is mirrored in the adolescent's attitude and behavior, both at home and at school (Mogonea and Mogonea, 2014). The adolescents with a high level of self-esteem have the following characteristics: they are capable of influencing positively the opinion and behavior of others; they tackle new situations positively and confidently; they have a high level of tolerance towards frustration; they accept early responsibilities, they assess correctly situations; they communicate positive feelings about themselves; they succeed in having a good self-control and the belief that the things they are undergoing are the result of their own behavior and actions (Lavoie, 2012). Therefore, adolescence is the critical period for the development of self-esteem and self-identity, and low self-esteem may endanger adolescent's emotional regulation (Lin, Tang, Yen, Ko, Huang, Liu et al., 2008). On the other hand, high self-esteem serving as a role of resilience or positive adaptation (Moksnes and Espnes, 2012).

ASSESSING SELF-ESTEEM

There are many measures of self-esteem, i.e., state and trait; explicit and implicit; uni- and – multi-dimensional scales. Because the Rosenberg scale is the most widely measure in research (Blascovich and Tomaka, 1991), and it is the most widely used measure of global self-esteem (Heatherton and Wyland, 2003), as well as other advantages, it is the scale of choice to introduce here.

Rosenberg Self-Esteem Scale (1965, 1987) (See Table 1)

It is the most popular measure of global self-esteem. Indeed, it is the standard with which developers of other measures usually seek convergence. It is unidimensional scale and face valid. It contains only 10 Likert type items contributes to ease of administration, scoring, and interpretation. The measure’s relatively high internal consistency and test-retest reliability undoubtedly contribute to its popularity. Possible susceptibility to social desirability effects has not dampened its use, probably because of similar problems with other scales. Although originally developed for use with adolescents, this scale is also used widely with adults (Blascovich and Tomaka, 1991).

Table 1. Rosenberg Self-Esteem Scale

1- Strongly Agree	2- Agree	3- Disagree	4- Strongly Disagree
1- I feel that I am a person of worth, at least on an equal basis with others.			
2- I feel that I have a number of good qualities.			
3- All in all, I am inclined to feel that I am a failure. *			
4- I am able to do things as well as most other people.			
5- I feel I do not have much to be proud of. *			
6- I take a positive attitude toward myself.			
7- On the whole, I am satisfied with myself.			
8- I wish I could have more respect for myself. *			
9- I certainly feel useless at times. *			
10- At times I think I am no good at all. *			

* Reverse-scored item

Note- many researchers change the 4-point scale to 5

Schmitt and Allik (2005) stated that the Rosenberg Self-Esteem Scale has been translated into 28 languages. They administered this scale to 16, 998 participants across 53 nations, and found that the factor structure of the scale was largely invariant across nations. Its scores correlated with neuroticism, extraversion, and romantic attachment styles within nearly all nations, providing additional support for cross-cultural equivalence of the scale. The negatively worded items were interpreted differently across nations.

However, there are many critics of the Rosenberg scale. Some consider it too short, too obvious, unrealistically unitary, and fails to predict behavior effectively (Baumeister, 1994). Moreover, Carmines and Zeller (1974) identified separate “positive” and “negative” factors related to the items worded in a positive direction and the items that worded in the negative direction, respectively, thereby suggesting response set.

Cheng and Hamid (1995) pointed out to a problem in translating the Rosenberg’s (1965) Self-esteem Scale into Chinese. That is, the negatively phrased item number 8: “I wish I could have more respect for myself”, was syntactically problematic in Chinese. When this item was translated into Chinese, it could imply either that (a) the person already has adequate self-esteem, yet wishes to have more or (b) the person has little respect for himself and would like more. The same problem is relevant in the Arabic language. For this reason, the five negatively phrased items converted to positively worded in the Arabic translated version (See: Abdel-Khalek, 2007; Abdel-Khalek, Korayem and El-Nayal, 2012).

The main problem in self-esteem questionnaires, including the Rosenberg scale, is the social desirability, i.e., the defensive self-esteem, denying personal problems and inadequacies, and making a false good impression.

POSITIVE EFFECTS OF HIGH SELF-ESTEEM

The self-esteem is a personality trait related with the self-image and self-conscience concepts. All personality traits, including self-esteem, could be viewed as a continuum or a bipolar dimension. Individual differences through this continuum encompass several grades and levels. Self-approval is a basic human desire. High self-esteem score is a must factor in order to attain the feeling of happiness.

Studies have identified self-esteem as an important determinant of emotional well-being (Baumeister, Campbell, Krueger and Vohs, 2003). As early as 1890, William James, one of the founding fathers of Western

psychology, argued that self-esteem is an important aspect of mental health. People with high self-esteem experience more happiness, optimism, and motivation than those with low self-esteem, as well as less depression, anxiety, and negative mood.

Using several Arab participants, it was found that self-esteem significantly and positively associated with love of life, mental health, satisfaction with life, happiness, and hope (Abdel-Khalek, 2007, 2011, 2012, 2013, Abdel-Khalek and Snyder, 2007). Persons with high self-esteem are more likely to persist in the face of difficult tasks than are low self-esteem persons (Baumeister et al., 2003). High self-esteem people are more resilient to the vicissitudes of life.

A high level of self-esteem supplies individuals with the ability to accept happy moments, to handle unpleasant situations, to cope effectively with challenges, to engage in close relationships and to improve their strengths. High self-esteem is also considered to positively moderate the expression of dysfunctional schemata and depressive symptoms at the experience of negative life events (Stavropoulos, Lazaratou, Marini and Dikeos, 2015).

Several studies have found that high self-esteem individuals are more persistent in the face of failure than the low self-esteem individuals. High self-esteem individuals also appear more effective in self-regulating goal-directed behavior (Di Paula and Campbell, 2002). Self-esteem is important for self-regulation and quality of life, and the relevance of self-esteem for positive psychology. Self-esteem provides the energy to mobilize human behavior as well as contributing to its direction (Mackinnon, 2015, p. 18).

Previous research has shown that self-esteem rises when a person succeeds, is praised, or experiences another's love, making self-esteem dependent on not only one's perceptions of himself but also other's perceptions of him (Schmidt and Padilla, 2003). Self-esteem is a major key to success in life. The development of healthy self-esteem is extremely important for good personal and social adjustment.

NEGATIVE EFFECTS OF LOW SELF-ESTEEM

People with low self-esteem suffer from feelings of worthlessness, inferiority, and emotional instability, so leading to dissatisfaction with life (Ha, 2006). Moreover, there is a tendency of respondents with low self-esteem scores to have a general negative attitude toward many things, including other people and personal circumstances (Mackinnon, 2015, p. 15).

Low self-esteem has been linked to depression, aggression, less competency to overcome difficulties and decreased level of well-being in adolescence (Stavropoulos et al., 2015). Weber (2001) hypothesized that college students who report emotional abuse are expected to have a lower self-esteem than those who do not report emotional abuse. This hypothesis was generally supported for male participants only.

Self-esteem was significantly and negatively correlated with internet addiction (Aydin and Sari, 2011). Using a cross-sectional convenient national sample of 23,532 Norwegians, Andreassen, Pallesen, and Griffiths (2016) elucidated the addictive use of social media in reflecting a need to feed the ego, i.e., narcissistic personality traits, and an attempt to inhibit a negative self-evaluation, i.e., self-esteem.

The sociometer model maintained that subjectively experienced self-esteem serves as “a psychological gauge or indicator” that allows people to efficiently monitor other’s reactions to them. Low self-esteem is conceptualized as an experiential indicator of social rejection (Leary, Schreindorfer and Haupt, 1995; Leary, Tambor, Terdal and Downs, 1995).

LOW SELF-ESTEEM AND PSYCHOPATHOLOGY

The low self-esteem person is an individual whose global self-evaluation is neutral, whose self-concept is uncertain and confused, who is highly susceptible to, and dependent on, external self-relevant cues, and whose social perceptions and behaviors reflect a cautious or conservative orientation (Campbell and Lavalley, 1993).

Low self-esteem can be understood in terms of confusion or uncertainty in self-knowledge, a cautious and self-protective approach to life, a shortage of positive resources in the self, and a chronic internal conflict. They lack a clear, consistent unified understanding of who they are, which leaves them at the mercy of events and changing situations (Baumeister, 1993).

Lowered self-esteem frequently accompanies psychiatric disorders. It has been suggested that low self-esteem is an etiological factor in many psychiatric conditions as well as in suicidal individuals. With 957 psychiatric patients, Silverstone, and Salsali (2003) found that all psychiatric patients suffer some degree of lowered self-esteem. The lowest self-esteem was found in patients with major depressive disorder, eating disorders, and substance abuse. The authors concluded that there is a vicious cycle between self-esteem and onset of psychiatric disorders (See also Murphy et al., 2005).

Based on three studies, Donnellan, Trzesniewski, Robins, Moffitt, and Capsi (2005) found a robust relation between low self-esteem and externalizing problems (aggression, antisocial behavior, and delinquency). This relation held for measures of self-esteem and externalizing problems based on self-reports, teacher's ratings, and parents' ratings, and for participants from different nationalities (United States and New Zealand), and age groups (adolescents and college students). Using a sample of 1,209 Norwegian adolescents, Moksnes and Espnes (2012), found that self-esteem was strongly and negatively associated with both state depression and state anxiety.

Low level of self-esteem has been linked to behavioral problems and poor school performance as well as serious behavioral problems as suicidal tendencies, maladjustment, and leads to psychological problems such as depression, social anxiety, loneliness, alienation, etc (Sharma and Agarwala, 2015).

Kempke, Luyten, Houdenove, Goossens, Bekaert, and Wambeke (2011) studied a sample of 192 patients with chronic fatigue syndrome. They found that self-esteem fully mediated the relationship between maladaptive perfectionism and depression. This finding may have important implications for the treatment and prevention of depression in these patients. As for the prognosis, longitudinal studies suggest that low self-esteem predicts depression later in life (Kamkar, Doyle and Markiewicz, 2012).

The most prominent explanations for the link between low self-esteem and psychopathology are as follows: the vulnerability model (low self-esteem increases the probability of psychopathology), and the scar model (low self-esteem is a consequence of psychopathology rather than a cause) (Zeigler – Hill, 2011).

SELF-ESTEEM AND DEPRESSION

Depression is used to describe a range of experiences from a slightly noticeable and temporary mood decrease, as well as a mild of tiredness and simple sadness, to the most profound state of apathy and severe symptoms of anhedonia, depressed mood, as well as a set of correlated affective, cognitive, and somatic symptoms.

According to the DSM5, the essential feature of a major depression is a period of at least two week of suffering from depressed mood, loss of interest or pleasure, significant weight loss, insomnia or hypersomnia, psychomotor

agitation or retardation, fatigue, feelings of worthlessness, diminished ability to think or concentrate, and recurrent thoughts of death or a suicide attempt (APA, 2013, pp. 160 – 161). The core symptoms of depression are the same for children and adolescents as for adults.

Depression prevalence has increased in the last few decades, affecting younger age groups. According to WHO, depression is the fourth leading disease and by 2020 it is expected to reach the second place (Tripković, Roje, Krnic, Nazor and Karin, 2015). Further, it is projected to be the leading cause of the global burden of disease by the year 2030 (World Health Organization, 2008).

Childhood and adolescence depression have become a subject of considerable studies over the past few decades. The clinical features of child and adolescent depression are similar to those of adult depression. Children cannot achieve their full potential if they are affected by emotional or mental disturbances that hinder their success. Children experiencing emotional abuse may experience depression and having low self-esteem. Parental behavior has been reported to relate to a child's self-esteem and is known to be as one of the reactions or consequences of psychological maltreatment (Weber, 2001). An extensive body of evidence supports the role of family processes in the development, course and maintenance of depression in children and adolescents (Sander and McCarty, 2005).

Self-esteem has been implicated as a vulnerability factor in the onset of depression in the theoretical model of Beck (1967). Further, the role of self-esteem has been salient in Brown and Harris's (1978) psychosocial model of depression. To understand the link between low self-esteem and depression, Orth and Robins (2013) described several theoretical models concerning this link. They concluded that the available evidence provides strong support for the vulnerability model (low self-esteem contributes to depression), and weaker support for the scar model (depression erodes self-esteem). Moreover, the vulnerability model is robust and holds across gender, age, affective – cognitive versus somatic symptoms of depression.

Several studies have shown that increased negative thinking about the self is a central feature of depression. It has been thoroughly demonstrated that depressive people think negatively and report lower self-esteem than non depressed controls. Likewise, several studies have suggested a combined effect of low self-esteem (or negative cognitions about oneself) and family functioning on the emergence of adolescent depression (Stavropoulos et al., 2015). Statistically significant association between low self-esteem and clinically significant depression was found (Tripković et al., 2015) (See also:

Sharma and Agarwala, 2014). On the other hand, a positive view of the self, i.e., self-esteem, plays a role in buffering the relation between negative events and depressive symptoms.

THE DARK SIDE OF HIGH SELF-ESTEEM

There is a dark side of self-esteem. Baumeister, Smart, and Boden (1996) suggested that people with high self-esteem are more likely to be conceited, arrogant, or occasionally narcissistic. They expect to receive positive evaluations from others; if they are provided with negative feedback, a threatened ego motivates them to spend personal resources on coping with the negative evaluations. Neff (2011) pointed out that pursuit of high self-esteem can be problematic, can sometimes be counterproductive, and may involve puffing the self up while putting others down.

High scores on self-esteem scales can result from narcissism – a highly inflated, grandiose view of oneself and one's positive traits and competence, conjoined with a sense of entitlement. Although separate measures of narcissism and normal self-esteem correlate substantially, they have different outcomes. Narcissism predicts aggression while normal self-esteem does not (Mackinnon, 2015, pp. 14 – 15).

Some authors view narcissism as an extreme form of self-esteem, so exaggerated self-esteem leads to narcissism and the search for unconditional acceptance. Narcissists see themselves as the center of the universe; in the mirror they only see themselves reflected. They see themselves as if they were the only reality worthy of esteem, the rest being mere generators of their supposed grandeur and worth. Narcissists are always talking about their supposed greatness and worth, about their uncommon, marvelous experiences, their extremely interesting projects, about how much they have done to improve in life, and even to help others (Dolan, 2007, p. 71).

The essential feature of narcissistic personality disorder is a pervasive pattern of grandiosity, need for admiration, a grandiose sense of self-importance, a belief that they are superior, special, or unique (APA, 2013, pp. 669 – 670).

Gerrard, Gibbons, Reis – Bergan, and Russell (2000) studied the self-serving cognitions among people with high self-esteem who engage in unwise or less optimal and risky health behavior, such as drinking, smoking, and unprotected sex. They found that they often utilize a variety of self-serving cognitive strategies and defensive self-justification and reactance that protect

them fully acknowledging their vulnerability to the potential negative consequences of their behavior. For example, they minimize their estimates of personal risk and overestimate the prevalence of the risk behavior among their peers. High self-esteem persons employ these strategies to cope with the inconsistency between their behavior and their positive self-perception.

TERROR-MANAGEMENT THEORY OF SELF-ESTEEM

Terror management theory was the first empirically oriented theory to address the question: What is the psychological function of self-esteem ? Terror management theory posits that people are motivated to pursue positive self-evaluations because self-esteem provides a buffer against the omnipresent potential for anxiety engendered by the uniquely human awareness of mortality. Therefore, self-esteem serves as anxiety – buffering terror management function (Pyszczynski, Greenberg, Solomon and Arndt, 2004).

This existential theory intends to explain two basic tendencies that are thought to characterize human behavior: a desire to maintain a favorable self-image, i.e., high self-esteem, and, at the same time, a desire to promote the beliefs and values of one's culture. Self-esteem and culture worldviews function as anxiety buffers to protect the individual from the existential terror or anxiety that is endangered by awareness of the inevitability of death and the extinction of one's culture. It is through the mechanism of self-esteem that culture buffers or reduces existential anxiety associated with death and dying (Greenberg, Solomon and Pyszczynski, 1997).

Terror management theory posits both the universal and the cultural specific aspects of self-esteem. On the one hand, the need for self-esteem in the service of anxiety reduction is universal. On the other hand, the specific manner by which self-esteem is acquired and maintained depends on the demands of a particular social milieu at any given point in time (Wang and Ollendick, 2001).

CULTURE AND SELF-ESTEEM

Culture shapes various aspects of the self: how people view themselves, what they strive to be, and when they feel good (or bad) about themselves (Tsai et al., 2001). The North American and Western European view the self

as an independent, autonomous, self-contained entity composed of a unique set of attributes (traits, feelings, values, etc.). These attributes determine and explain a person's behavior (Marcus and Kitayama, 1991). In this culture, where individualism is prized, the culture urges individuals to view themselves as independently functioning agents. People who embrace an independent view of self tend to have a sense of identity that is anchored in its internal attributes and is viewed as the source of action and the center of control. Independent view of self and self-esteem are positively correlated (Heine and Lehman, 2004).

On the other hand, the cultural framework of interdependence characterizes many Asian, African, and Hispanic collectivistic cultures (Triandis, 1994). That is, the self cannot be separated from others and the surrounding social context. Thus, the goal is not to become separate but to fit in with others.

In an independent culture, high self-esteem derives from evidence that one has distinguished oneself from others and is different and unique in some positive sense. By contrast, in an interdependent culture, high self-esteem is a function of good social relationships, a sense of fitting in, belonging, and to minimize any sense of uniqueness or distinction (Glaeser, 1999; Marcus and Kitayama, 1994).

Interdependence are more strongly embraced by East Asians, particularly Japanese. Research indicated that the vast majority of North Americans report having high self-esteem. Japanese consistently have exhibited lower self-esteem scores than North Americans (Heine and Lehman, 2004). In a large scale study, the same authors have reached the conclusion that greater exposure to Western culture leads to higher self-esteem.

Using a Chinese American sample, Tsai et al. (2001) found that cultural orientation significantly predicted self-esteem above and beyond the contribution of age, gender, grade point average, and socioeconomic status. Although people from East Asian countries consistently report lower self-esteem than do those from Western countries, the origins of this difference are unclear. Cai, Brown, Deng, and Oakes (2007) found that Chinese participants appraised themselves less positively than American participants on a cognitive measure of self-evaluations, but cultural differences were absent on a measure of affective self-regard. They found also that cultural differences in modesty underlie cultural differences in cognitive self-evaluation.

Brown (2008) studied the beliefs about the importance and desirability of self-esteem. He found that American students view self-esteem as desirable and consequential, whereas Japanese students view self-esteem as desirable

but not consequential. In a similar vein, university students from non-Western culture (Nigeria and Nepale) tend to report higher academic but lower non-academic self-esteem than their Western peers (Watkins, Akande, Cheng and Regmi, 1996).

SELF-ESTEEM AND DEMOGRAPHIC VARIABLES

Some studies investigated self-esteem, socio-economic status, and urban-rural residence associations. Housley, Martin, McCoy, Greenhouse, Stigger, Chapin et al. (1987) found that for American urban girls, mean self-esteem of upper economic status subjects was significantly higher than that of those at the lower economic status. The self-esteem of upper economic status urban girls was significantly higher than the self-esteem of their rural peers.

Houlihan, Fitzgerald, and O'Regan (1994) recruited 464 Irish adolescents. They found that the females from the rural setting suffer significantly more depressive symptomatology (22%), and poor self-esteem (68%) than the male and urban participants. Tsai et al. (2001) stated that socioeconomic status has been found to correlate with self-esteem, with individuals of higher social class having more positive feelings about themselves than those of lower social class.

Males typically report higher self-esteem than do females. A number of studies suggest that boys and girls diverge in their primary source of self-esteem, with girls being more influenced by relationships and boys being more influenced by objective success (Heatherton and Wyland, 2003). Individuals who achieve academically have higher self-esteem than those who do not (Tsai et al., 2001). You, Shin, and Kim (2016) found that body image has significant effects on both self-esteem and depression.

Sharaf, Thompson, and Walsh (2009) studied the protective effects of self-esteem and family support on suicide risk behaviors among at-risk adolescents. They found that family support moderated that impact of self-esteem on suicide risk, and the ameliorating effect of self-esteem was stronger among adolescents with low versus high family support.

STABILITY OF SELF-ESTEEM

Most theories of self-esteem view it as a relatively stable personality trait. Based on this perspective, self-esteem is stable because it slowly builds over time through personal experiences, such as repeatedly succeeding at various tasks or continually being valued by significant others. A number of studies, however, assumed that self-esteem can momentarily manipulated or affected. Therefore, self-esteem can be viewed as a “trait” as well as a “state” (Heatherton and Wyland, 2003).

Greenier et al., (1995) stated that self-esteem instability is a dimension distinct from level of self-esteem. Self-esteem instability refers to the magnitude of short term fluctuations that people experience in their contextually based feelings of self-worth. Among high self-esteem individuals, self-esteem instability reflects fragility in one’s positive self-feelings, and is associated with heightened tendencies to defend and promote these positive self-feelings. On the other hand, among low self-esteem individuals, self-esteem instability is related to various indices of psychological difficulties and maladjustment.

CONCLUSION

Thousands of research papers have published in the self-esteem subject and many normal and abnormal variables have been correlated with it. Indeed, much knowledge about self-esteem has been accumulated in the mainstream Western industrialized culture. A less amount of research papers has been published to compare Western and Asian, particularly Chinese and Japanese samples, on self-esteem. However, the studies with the Arab participants are scarce. The Arab countries have special characteristics regarding geography, history, political conditions, economy, language, religions, culture, and child rearing practices, among other factors. Further studies are needed to examine the level, the underlying meanings, and the social and cultural contexts that influence self-esteem among Arab participants, as well as its cultural predictors.

Regarding the assessment issue, a major problem inherent in the measures of self-esteem is the extent to which self-reports are influenced by self-presentational concerns (Heatherton and Wyland, 2003). The available measures seem to be contaminated with social desirability.

As Baumeister (1994) stated, people are guided by a strong, fundamental desire for self-esteem. People claim credit for success but deny blame for failures. They exaggerate and overestimate their abilities ... they systematically choose to compare themselves with others who are less gifted or worse off than themselves. They deny or conceal their shortcomings and advertise their virtues and positive traits. They identify themselves with successful groups and distance themselves from failing or stigmatized groups (p. 84).

One solution to face the problem of responding to the self-esteem measures according to social desirability, might be to use measures of defensiveness to tease out the variance related to the self-report biases, such as the Crowne and Marlowe Scale (1960). It is suggested to administer a social desirability scale along with the self-esteem questionnaire. Then, it is hopeful to reach a statistical formula to add a score of the social desirability scale to the self-esteem measure to suppress the participant's attempt to make favorable impression. This proposed procedure is similar to the mathematical correction of some clinical scales of the MMPI for defensiveness by adding a fraction of the K and L validity scores to the clinical scales.

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Self-Esteem

*Perspectives, Influences
and Improvement
Strategies*



Psychology of Emotions,
Motivations and Actions

— Franklin Holloway —
Editor

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PSYCHOLOGY OF EMOTIONS, MOTIVATIONS AND ACTIONS

SELF-ESTEEM
PERSPECTIVES, INFLUENCES AND
IMPROVEMENT STRATEGIES

FRANKLIN HOLLOWAY
EDITOR



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